



NASOGASTRIC TUBE (NGT) CONFIRMATION of POSITION on INSERTION

The decision to insert NGT must be taken jointly by doctor responsible for patient and nurse involved in the patient's care. The decision must be documented in the patient records. Insertion and confirmation of position should only be undertaken by staff deemed competent according to UCLH Trust Policy. UCLH Policy recommends to use a fine bore NG tube for administration of enteral nutrition and medication.

FIRST LINE TEST

GASTRIC ASPIRATE pH

Target insertion pH ≤ 4.5

NGT can be aspirated with guide wire in place

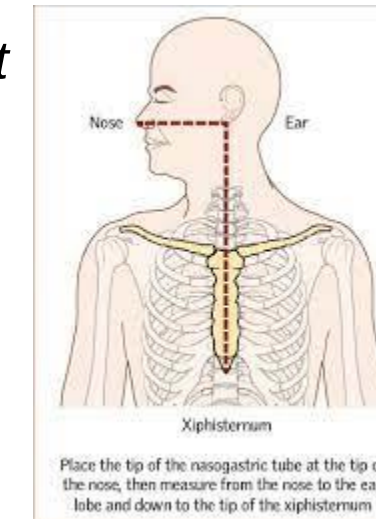
2 COMPETENT CHECKERS REQUIRED AND
pH MUST BE RECORDED AND SIGNED

Prior to obtaining gastric aspirate to confirm placement flush NGT with 10mL air to reduce false positives from liquid residing in tube from insertion or previous use

THIS SHOULD NOT BE CONFUSED
WITH THE NOW BANNED
"WHOOSH" TEST

Estimate required length prior to insertion using NEX MEASUREMENT

Place exit port of tube at
Tip of **Nose**
Extend tube to **Earlobe**
Then to **Xiphisternum**



If NGT inserted too far obtaining aspirate for daily pH checks will be difficult

CEASE SIGNS

THIS PATIENT HAS A NASOGASTRIC TUBE (NGT)
Triggers to immediately CEASE NG feed and medications until correct positioning of NGT is reconfirmed

C	Coughing spasms / Frequent suctioning
E	Elevated respiratory rate / Difficulty breathing
A	Agitation / Drop in GCS
S	Saturation drop / Increasing O ₂ requirement
E	Elevated heart rate / Chest pain

If any **CEASE** symptoms are present (especially following recent insertion of NG tube) inform nurse in charge immediately. Refer to NG Protocol / Checklist and Trust Handbook 'Enteral Nutrition Support for Adults'.

REMOVE NGT IMMEDIATELY IF PATIENT HAS ANY ADVERSE SYMPTOMS DURING OR FOLLOWING PLACEMENT

- Fall in oxygen saturation
- Increased respiratory rate
- Difficulty breathing
- Chest pain / arrhythmia

See **CEASE** signs above

NG TUBES IMPORTED FROM OTHER CLINICAL AREAS

PRIOR TO USE

1. Check insertion documented
2. Check NEX measurement
3. Check gastric aspirate ≤ 4.5

pH ≤ 4.5

YES

Complete
NGT Insertion
Checklist &
commence
using NGT

SUBSEQUENT CHECKS

Reconfirmation of position every 24h –
record insertion length / pH

Target pH ≤ 5.5

- pH of 5.5 acceptable if on PPI or continuous feed
- If pH >5.5 consider stopping feed for **one hour** to allow pH to fall - caution if on **insulin infusion**
- Recheck if disconnected from feed for length of time – physio, CT scan

TRIGGERS FOR MORE FREQUENT CHECKS

Vomiting
Coughing spasms
Frequent suctioning
Respiratory symptoms
Fall in oxygen saturation
Any concern about position

See **CEASE** triggers

pH >4.5

YES

Do **NOT** use NGT
Wait **1 hour**
then recheck pH

pH >4.5
after 1 hour

NO

YES

Do **NOT** use NGT
Continue to wait to
allow pH to fall

pH >4.5
after 4 hours

NO

YES

Do **NOT** use NGT
Seek advice from NCC
Nurse in Charge & NCC
doctor whether to
wait longer to allow
pH to fall – for further
4 hours maximum
or
to proceed to x-ray

Unable to obtain gastric aspirate

Do **NOT** use NGT

- Lie patient on L side
- Flush NGT with 20ml air to clear
- Wait **1 hour** & try again
- Check NGT still at desired length
- Advance 10-20cm

Aspirate Obtained?

YES

pH ≤ 4.5

YES

Complete
NGT Insertion
Checklist &
commence
using NGT

NO

Do **NOT**
use NGT
Wait **1 hour** &
recheck pH

SECOND LINE TEST

X-RAY

- Limit 'out of hours' requests
- Radiology report in 1 hour (Reporting out of hours may take longer)
- Final decision whether safe to commence feeding lies with the clinical team responsible for patient
- Clinical team review x-ray report & document
 - ✓ NGT passed down midline
 - ✓ Past level of diaphragm
 - ✓ Deviates to left
 - ✓ No tube in lung fields
 - ✓ Safe to use NGT

UCLH GUIDELINES 2021

